# The Person Behind the Care

A short guide to writing down Dad's preferences while things are calm.

The second cup of coffee. The radio station. The walk after lunch. Dad wouldn't think any of it worth mentioning, and you might not either, until you tried to describe his day to someone who had never met him.

This guide explains why those small details are worth writing down, why they're kept apart from medical instructions, and how a family goes about it when there's time. It explains. It doesn't decide anything, and it doesn't assume anything is about to change.

## Why the small things matter

Two federal sources treat everyday preferences as real information. The Administration for Community Living describes person-centered planning as a process directed by the person who receives the support, and it counts preferences about recreation, friendships, family relationships and social activities among the things that planning takes in. The federal guide for nursing home residents says a home must recognize and respect a resident's preferences when giving care, and that residents have the right to make their own schedule, like when they go to bed, wake up and eat meals. Nobody is talking about any of that for Dad. It's here because it shows what the details of an ordinary day are worth, and the only person who can supply them is him. [1] [2]

## Listen while there's nothing to decide

An ordinary week is a good time to ask. AARP's planning guide for families admits that conversations like this are easy to put off, because it never seems like the right time, and adds that you may be surprised to find your loved one has been meaning to have the talk too. It expects the conversation to take place over time, and its advice for it is short: be open and, most importantly, listen. [3]

The Conversation Project's workbook, written to help people tell their health care team what is most important to them, says much the same to anyone helping another person through its questions: take it in small pieces, it's always okay to skip a question, and if the person gets tired, take a break and come back to it later. [4]

- Use the conversations you already have: a meal, a drive, the phone call on Sunday.
- Read your notes back and invite corrections. “Did I get that right?” is the most useful question you have.
- Keep the page loose. A date at the top and room in the margins tell everyone it can change.

## Learn his ordinary day

The answers will be small, and they're meant to be. The Conversation Project's workbook asks “What does a good day look like for you?” and its own examples are get up and dressed, play with my cat, make a phone call, watch TV, have coffee with a friend. A form the Alzheimer's Association publishes for telling others about a person asks for a typical day and for any established routines, “such as having coffee and newspaper in the morning.” That form was written with dementia care in mind. We've borrowed only its questions about an ordinary day. [4] [5]

Ask open questions and let him tell it as a story. “What's a good morning for you?” gets you more than “Do you still get up at six?”

- Write his phrases, not your summary. “Eggs soft, toast dark, and the radio on low” says more than “likes a quiet breakfast.”
- Include the routines and the people: who he calls, who drops by, what happens every Thursday.
- Note the parts of the day you didn't get to, so you know what to ask next time.

## Keep a preference a preference

Some of what a family “knows” about Dad's food may not be a preference at all. MedlinePlus, from the National Library of Medicine, describes a food allergy as an abnormal immune system reaction to certain foods. That is a medical matter, not a like or a dislike. The Alzheimer's Association form keeps the two apart on the page, with separate lines for special dietary needs, food allergies, favorite foods and strong dislikes. [6] [5]

So keep separate lists: what he likes and dislikes, any allergy the family has been told about, and anything a doctor has told him to eat or avoid. You don't have to work out which is which. Write down who said it and where it's written, and take the question to his doctor or pharmacist.

The same care applies to opinions. What Dad prefers and what the family thinks is best for him are two different things, and both can go on the page as long as each is labeled. A fact sheet for families of nursing home residents, from the National Consumer Voice, says a resident's preferences may be different than what you think is best, but the resident's needs and wishes should always be respected. AARP's guide says to try not to anticipate what your loved one might say. [7] [3]

- Where he corrects you, keep his version and note yours as background.
- For the things he enjoys doing, write down what he says it takes to keep doing them: the ride to the Thursday card game, the friend who walks with him.
- Don't turn a preference into a rule. “He likes dinner at six” is a note about him. It isn't an instruction for anyone.

## A page that's easy to read and easy to change

What you're working toward is a short page about Dad that someone who has never met him could take in quickly. The Family Care Roadmap calls the finished version an About Me profile. This guide stops well short of that. A dated page of notes in his words is the family's part.

Two things belong on it from the start. The first is his say over who reads it. The Conversation Project's workbook asks people how much they want those close to them to know, on a scale that runs from not all the details to all of them, so ask him and write down what he says. The second is a date. The workbook notes that people's preferences often change as their health changes or as time goes by, and suggests revisiting the answers. The federal guide for nursing home residents says the same of that setting: preferences and goals may change, so the home should have ongoing discussions with the resident about updating the care plan. [4] [2]

- Put “as of” and the date at the top, and who wrote it at the bottom.
- Agree with Dad on when you'll go over it again.
- Leave the finished version for later. Nothing calls for it now.

## What this means for your family

- The small details are the content, and nobody but Dad can supply them.
- Your job is to ask, listen and write down his words, with a date. It isn't to decide what he'd want.
- Likes and dislikes go on one list. Allergies and anything a clinician has told him go on another, and questions about those go to his doctor or pharmacist.
- The page is a draft on purpose. Today's answers are today's.

**What this guide doesn't do**

It doesn't explain any medical condition or diet, say what Dad would want in a situation he hasn't talked about, or produce his finished profile. The quoted material comes from the sources below. Some of them were written for nursing home residents or with dementia care in mind; we've drawn only on what they say about everyday routines and preferences, and nothing here assumes either applies to Dad. The advice about what to write down is ours.

## Ready for the next step?

You understand now why the small details of Dad's day are worth writing down, and why they stay apart from medical instructions. The care interview keeps them that way: it takes his preferences alongside the rest of his situation, one area at a time, and turns your answers into a Family Care Profile you can bring to any care conversation.

**Create Your Free Family Care Profile ›**

### WHERE THIS COMES FROM


1. Administration for Community Living, Person-Centered Planning
2. Centers for Medicare & Medicaid Services, Your guide to living in a nursing home (CMS Product No. 12217)
3. AARP, Prepare to Care: A Planning Guide for Families
4. The Conversation Project (Institute for Healthcare Improvement) and Ariadne Labs, What Matters to Me: A Workbook for Better Health Care
5. Alzheimer's Association, Personal Facts and Insights (form)
6. MedlinePlus (National Library of Medicine), Food Allergy
7. National Consumer Voice for Quality Long-Term Care, Family Involvement in Nursing Homes (fact sheet)



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